

Khalitya and Palitya

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खालित्य (Khālitya)

तेजोनिलौः सह केशभूमिं दग्ध्वाऽऽशु कुर्यात् खलति नरस्य ।
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Baldness of head due to loss of scalp hair is known as *Khālitya* or *Khalita* and it is a disease of scalp. It is caused by vitiation and aggravation of *pitta* and *vāta doṣa* together and cause *dagda* or burning of scalp (*keśa bhūmi*) leading to the hair fall due to morbidity in the hair follicles.

रोमकूपानुगं पित्त वातेन सह मूर्च्छितम् ।
प्रच्यावयति रोमाणि ततः श्लेष्मा सशोणितः ॥
रुणद्धि रोमकूपांस्तु ततोऽन्येषामसंभवः ।
तदिन्द्रलुप्तं खालित्यं रुज्येति च विभज्यते ॥

S.S.Ni. 13/33-34

Exacerbated and vitiated *Pitta* and *vāta* reaches *romakūpa* (pore of the skin/hair follicle) and burns the scalp skin and demolish *romakūpa* leading to hair fall. Intensified *Kapha* and *shonita* causes thickening of the scalp due to their impaired the qualities resulting into the blockage of hair follicles and causes no more new hair growth.

Indralupta

The falling of the scalp hair is also known as *Indralupta*. The difference between *Indralupta* and *Khālitya*

Indralupta	Khālitya
Patchy hair loss in the scalp	Huge area of scalp is involved
Loss of hairs is sudden	Loss of hairs is gradual

वातज खालित्य (Vātaja Khālitya)

The Skin of the scalp turns dry due to properties of *vāta*.

पित्तज खालित्य (Pittaja Khālitya)

It is associated with sweat (*Sweda*) glands in *romakūpa*.

कफज खालित्य (Kaphaja Khālitya)

In this case the Scalp becomes thick.

पालित्य (Pālitya)

शोकश्रमक्रोध कृतः शरीरोष्म शिरोगतः ।
केशान्सन्दोषः पचति पत्रितं सम्भवत्यतः ॥

Graying of scalp hair is called as *Pālitya* and it is usually develops due to ageing.

Hair Disorders

Patients may present with too much hair (hirsutism) or too little hair (alopecia)

Tinea capitis

- Fungal scalp infection is becoming increasingly common in urban areas in the UK, where it is most frequently spread by contact between children.
- Clinical features are variable due to associated inflammation with patchy-hair loss and some scaling.
- Affected individuals should have the area scraped and affected hairs plucked for mycological microscopy and culture.

Endothrix (within the hair shaft) infections

- Endothrix (within the hair shaft) infections, e.g. *Trichophyton tonsurans*, cause relatively uninfamed patchy baldness with breakage of the hairs at the skin surface ('black dot').
- There is no fluorescence under Wood's light.

Ectothrix (outside the hair shaft) infections

- Ectothrix (outside the hair shaft) species of fungi, such as *Microsporum audouinii* (anthropophilic), show minimal inflammation
- *Microsporum canis* (from dogs and cats) infections are more inflamed and can be identified by green fluorescence with Wood's light.

- Kerions are boggy, highly inflamed areas of tinea capitis and are usually caused by zoophilic (from animals, e.g. cattle ringworm) species of fungi (e.g. *Trichophyton verrucosum*).

Alopecia areata

- This non-scarring condition appears as sharply defined non-inflamed bald patches, usually on the scalp.
- During the active stage of hair loss, pathognomonic 'exclamation mark' hairs are seen (broken-off hairs 3-4 mm long, which taper off towards the scalp).
- An uncommon diffuse pattern on the scalp is recognised.
- The condition may affect the eyebrows, eyelashes and beard.
- Pitting and longitudinal wrinkling of the nail may be seen.
- The hair usually regrows spontaneously in small bald patches, but the outlook is less good with larger patches and when the alopecia appears early in life or is associated with atopy.
- There is an association of alopecia areata with autoimmune disorders, atopy and Down's syndrome.

Alopecia totalis

- It describes complete loss of scalp hair

Alopecia universalis

- It is complete loss of all hair.

Classification alopecia

Localised	Diffuse
Non-scarring	
Tinea capitis	Androgenetic alopecia
Alopecia areata	Telogen effluvium
Androgenetic alopecia	Metabolic
Traumatic (trichotillomania, traction, cosmetic)	Hypothyroidism
Syphilis	Hyperthyroidism
	Hypopituitarism
	Diabetes mellitus
	HIV disease
	Nutritional deficiency

	Liver disease Post-partum Alopecia areata Syphilis
Scarring Idiopathic	Discoid lupus erythematosus
Developmental defects	Radiotherapy
Discoid lupus erythematosus	Folliculitis decalvans
Herpes zoster	Lichen planus pilaris Pseudopelade

Androgenetic alopecia

1. Male-pattern baldness is physiological in men over 20 years old, although rarely it may be extensive and develop at an alarming pace in the late teens.
2. It also occurs in females, most obviously after the menopause.
3. The well-known distribution (bitemporal recession and then crown involvement) is described as 'male-pattern' but hair loss in females is often diffuse.

Investigations

The following tests help to determine the cause of non-scarring alopecia.

1. A full blood count
2. ESR
3. Urea and electrolytes
4. Liver and thyroid function tests.
5. Autoantibody profile
6. Syphilis serology

Specialised tests,

7. The hair pluck test in which up to 50 hairs are removed with epilating forceps to determine the anagen:telogen ratio
8. Mycological assessment is advisable in cases of localised hair loss with scaling.
9. In scarring alopecia, scalp biopsy, with direct immunofluorescence, may help to confirm a diagnosis of lichen planus of the scalp or discoid lupus erythematosus.

Hirsutism

- It is the growth of the terminal hair in a male pattern in females.
- It is one of the most common endocrine disorders affecting less percentage of women.
- The hairiness implies the presence of abnormal androgen action, which may represent a serious or, more likely, a nonserious medical problem.
- Regardless of the etiology, hirsutism can produce mental trauma and emotional anguish.

Causes

1. Endocrine-related causes
 - adrenocortical disorders and ovarian disorders
 - Tumors, Cushing syndrome, and CAH are the adrenocortical causes.
 - Tumors (malignant or benign) and PCOS are the ovarian causes of hirsutism.
2. Idiopathic
3. Medication-related
4. Miscellaneous.

MCQs

1. *Nakhaprabha* is the feature of
 - a. *Vataja Khalitya*
 - b. *Pittaja Khalitya*
 - c. *Kaphaja Khalitya*
 - d. *Sannipatika Khalitya*

2. *Agnidagdhabha* is the feature of
 - a. *Vataja Khalitya*
 - b. *Pittaja Khalitya*
 - c. *Kaphaja Khalitya*
 - d. *Sannipatika Khalitya*

3. *Swinna siravrita* is the feature of
 - a. *Vataja Khalitya*
 - b. *Pittaja Khalitya*
 - c. *Kaphaja Khalitya*
 - d. *Sannipatika Khalitya*

4. *दग्धाग्निनेवनिलोमासदाहा* is the feature of
 - a. *Vataja Khalitya*
 - b. *Pittaja Khalitya*
 - c. *Kaphaja Khalitya*
 - d. *Sannipatika Khalitya*

5. *जलप्रभम्* is seen in
 - a. *Pittaja Khalitya*
 - b. *Vataja Palitya*
 - c. *Vataja Khalitya*
 - d. *Kaphaja Khalitya*

Answer

1.	d	2.	a	3.	b
4.	a	5.	b		